RVMC Clinical Operations

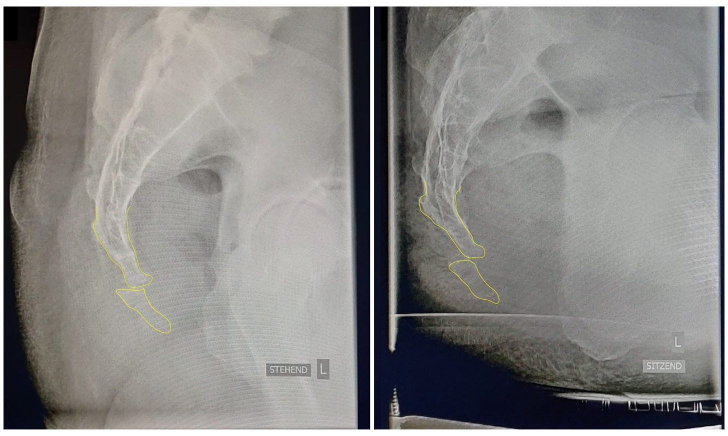
GPC Clinical Corner

July 26, 2023

Dr. Bruce R. Koyle

GROUP PRACTICE COORDINATOR

Coccydynia: Etiology and Treatment of Coccyx Pain



Note. From *Coccyx laterally standing and sitting with pathological mobility* [X-Ray], by @ToRaToRa71, 2022, Wikimedia (https://commons.wikimedia.org/wiki/File:Stei%C3%9Fbein_seitlich_im_Stehen_und_Sitzen_mit_krankhafter_Beweglichkeit.png).

Common causes of tailbone (coccyx) pain include:

- Pregnancy and childbirth
- An injury or accident, such as a fall onto the coccyx
- Repeated or prolonged strain on the coccyx (e.g., after sitting for a long time while driving or cycling)
- Joint hypermobility (increased flexibility) of the joint that attaches the coccyx to the bottom of the spine
- Being overweight or underweight
- Poor posture
- Sometimes, the cause of tailbone pain is unknown

Etiology

The exact incidence of coccydynia has not been reported; however, factors associated with increased risk of developing coccydynia include obesity and being of the female sex. Women are 5 times more likely to develop coccydynia than men. Adolescents and adults are more likely to present with coccydynia than children. Anecdotally, rapid weight loss can also be a risk factor because of the loss of mechanical cushioning. The most common etiology of coccydynia is external or internal trauma.

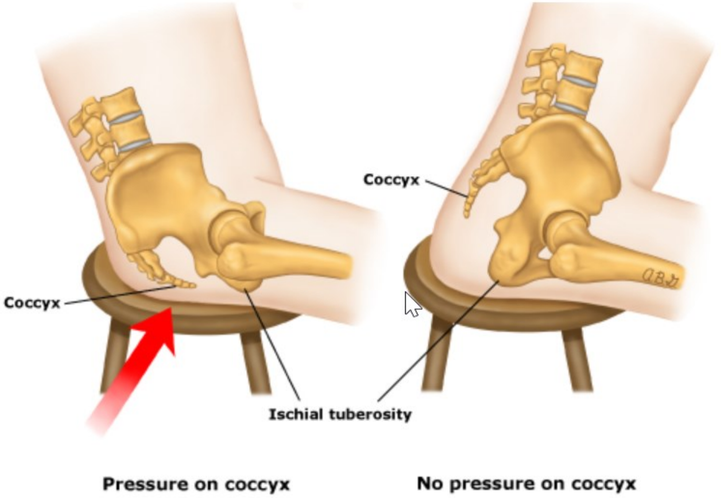
Diagnosis

The classic presentation of coccydynia is localized pain over the coccyx. Patients present complaining of “tailbone pain.” The pain will usually be worse with prolonged sitting, leaning back while seated, prolonged standing, and rising from a seated position. Pain may also be present with sexual intercourse or defecation. History may be significant for a recent trauma with an acute onset of pain, or the onset of pain may have been insidious with no clear inciting factor.

Physical examination will reveal tenderness over the coccyx. Rectal examination allows the coccyx to be grasped between the forefinger and thumb. Manipulation will elicit pain and may reveal hypermobility or hypomobility of the sacrococcygeal joint. Normal range of motion should be approximately 13 degrees. Other causes of coccyx pain—such as infection etiologies (e.g., pilonidal cyst), masses, and pelvic floor muscle spasms—should be ruled out.

Treatment

Multiple conservative treatment options are available for coccydynia. Conservative treatment is successful in 90% of cases, and many cases resolve without medical treatment. Relatively simple measures are sufficient in most cases. Modified wedge-shaped cushions (coccygeal cushions) can relieve the pressure on the coccyx while the patient is seated and are available over the counter. Circular cushions (donut cushions) have been suggested for the treatment of coccydynia, but they can place pressure on the coccyx by isolating the coccyx and ischial tuberosities. They are more useful for treating rectal pain.



Note. From *Seating Position and Coccydynia (Pain in the Coccyx)* [Infographic], by UpToDate, 2023, UpToDate (https://sso.uptodate.com/contents/image?imageKey=PC%2F59325).

Poor posture can contribute to coccydynia; training patients to adopt proper sitting posture can correct this. Applying heat and cold over the site may also be beneficial. Patients should try both, as one has not been shown to be superior to the other. Nonsteroidal anti-inflammatory drugs (NSAIDs) are the most common analgesics prescribed for coccyx pain; opioids generally are not recommended and are reserved for rare cases of severe pain—usually from an acute injury—that is not responsive to other measures.

Conclusion

Coccydynia is a common condition that is often self-limited and mild. Although most patients who seek medical attention respond to conservative treatments, some patients require more aggressive treatments. In these cases, the etiology of the coccydynia may be complex and multifactorial. A multidisciplinary approach, employing physical therapy, ergonomic adaptations, medications (NSAIDs), injections, and, possibly, psychotherapy leads to the greatest chance of success in these patients. Surgical coccygectomy generally is not recommended, and although different surgical techniques are emerging, more research is needed before their efficacy can be established.

References

@ToRaToRa71. (2022). Coccyx laterally standing and sitting with pathological mobility [X-Ray]. Wikimedia. https://commons.wikimedia.org/wiki/File:Stei%C3%9Fbein_seitlich_im_Stehen_und_Sitzen_mit_krankhafter_Beweglichkeit.png

Lirette, L. S., Chaiban, G., Tolba, R., & Eissa, H. (2014). Coccydynia: An overview of the anatomy, etiology, and treatment of coccyx pain. *The Ochsner Journal, 14*(1), 84-87. https://www.ncbi.nlm.nih.gov/pmc/articles/PMC3963058/

National Health Service. (2022). Tailbone (coccyx) pain. NHS. https://www.nhs.uk/conditions/tailbone-coccyx-pain/

UpToDate. (2023). Seating position and coccydynia [Infographic]. UpToDate. https://sso.uptodate.com/contents/image?imageKey=PC%2F59325